جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : فاتن علي السيد عبدالقادر
المحافظة : الجيزه **الحي / المدينة :** سته اكتوبر **العنوان :** زايد
رقم البطاقة : 25610100104609 **رقم التليفون :** 01100062138 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 10-10-1956 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان البنكرياس
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان البنكرياس
خطة العلاج : أورام كيماوي علاج تدعيمي وتلطيفي [6287] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: فاتن على السيد عبدالقادر
National ID: 25610100104609

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

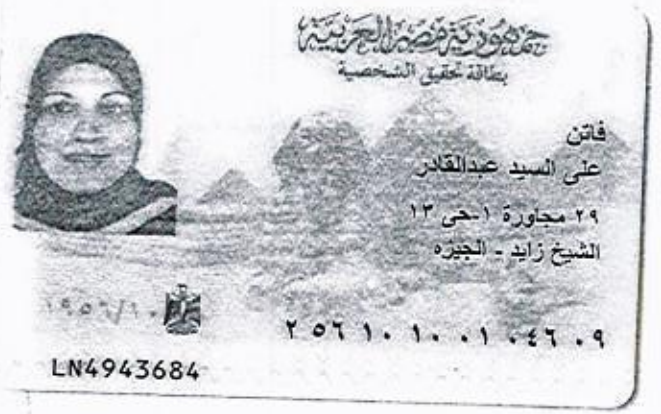
نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

A patient of pancreatic cancer for Supportive and palliative Treatment

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين



National ID – Front
[[DOCTYPE:ID_FRONT]]



National ID – Back
[[DOCTYPE::ID_BACK]]

USTAVE/
OUSSY

Patient Name	: الأستاذه / فاتن على السيد عبد القادر	Request No.	: 13001352
Patient No.	: 2327-2025	Request Date	: 24/09/2025
Path No.	:	Verification Date	: 24/09/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 68 Y	Department	: عيادات خارجية
Referreing Doc	:	Registered by	: Amr Hussein

Bone Marrow Report

Bone Marrow Aspiration Report

*BMA (Bone Narrow Aspiration) differential count:

Normoblast	: 30
Blast	: 0
Promyelocyte	: 23
Meylocyte	: 9
Juvenile	: 2
Staff	: 7
Segmented	: 7
Lymphocytes	: 3
Monocytes	: 3
Eosinophils	: 1
Basophils	: 0
Plasma Cells	: 15

*PB Comment: leucopenia, anemia with raulaux, normal platelets.

*BMA Comment:

-Cellularity: High normocellular marrow for age

-Hematopiesis: Erythroid hyperplasia, Normal granulopoiesis, and Normal Thrombopoiesis.

-Normal lymphopoiesis.

-Abnormal Infilteration: increased plasma cells with binucleated and multinucleated forms.

-Others:

*Conclusion:

Referred patient as M.M follow up, High normocellular marrow for age, increased plasma cells with binucleated and multinucleated forms.

*Recommendations: Correlation with Serum protien electrophoresis, immunofixation, BMB and marker study.

Signature:
Dr.Hend Abd Elhady, M.D
Ass. Prof. of Hematopathology, NCI

Verified by :
Dr.\Hassan Fadl

Bone Marrow Aspiration
[[DOCTYPE:BONE_MARROW_ASPIRATION]]

Patient Name	: الأستاذه / فاتن على السيد عبد القادر	Request No.	: 13001404
Patient No.	: 2327-2025	Request Date	: 24/09/2025
Path No.	:	Verification Date	: 05/10/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 68 Y	Department	: عيادات خارجية
Referreing Doc	:	Registered by	: Dr.\Hassan Fadl

Bone Marrow Report

BMB (Bone Marrow Biopsy) Report

Clinical data:
Multiple myeloma follow up
BMA comment:
High Normocellular for age, Normal hematopoieis, Plasma cells 15%.
Gross examination:
BMB core from R/PSIS, 1.5 cm, unilateral,
Technique: Sections were prepared from parrafin block, processed in Bench Mark ultra machine (Ventara),and stained with rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. the sections treated against: CD138 (B-A38)
Microscopic examination:
Adequacy: adequate more than 7 intertrabecular spaces.
Cellularity: Hypercellular for age (70%) cells: fat spaces.
Hematopoiesis: Normally represented with normal granulopoieis in count and maturation, erythroid hyperplais with erythroid nests, normal megakariopoiesis in count, maturation and topography.
M/E ratio: decreased
Fibrosis: MF grade 0/III
Abnormal Infilteration: small patchy and Interstitial plasma cells
IHC: -
CD138 = 10-15% positive
Diagnosis and Conclusion:
Referred patient as M.M follow up, Hypercellular marrow for age, showing increased plasma cells 10-15% highlighted with CD138.
Recommendations:Correlation with protien electrophoresis, immunofixation, free light chain assay.

Dr.Hend Abd Elhady, M.D

Verified by :
Dr.Asmaa Hamed

Bone Marrow Aspiration
[[DOCTYPE:BONE_MARROW_ASPIRATION]]

Patient Name	**: فاتن على السيد**	**Patient Id**	**: 1905**
Patient Info.	**: Female 69Y**	**VisitId**	**: 359561**
Department	**: Out Patient**	**Collection Date**	**: 22/08/2026 09:45:12**
Profile NO.	**: 2327-2025 /25610100104609 / 11891**	**Reporting Date**	**: 03/09/2026 12:55:31**

Bone Marrow Biopsy

Clinical Data :

Referred patient as MM .

Peripheral blood comment :

Pancytopenia

Bone marrow aspiration comment :

Hypocellular for age ,5% plasma cells

Nature of specimen :

Bone marrow biopsy

Gross Examination :

BMB core from R/PSIS, 1.5 cm, unilateral

Technique :

Sections were prepared from paraffin block, processed in Bench Mark ultra-machine (Ventara), and stained with rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain.
the sections treated against:

Microscopic Examination.

Adequacy :

Adequate more than 7 inter-trabecular spaces.

Cellularity :

Marked Hyper cellular for age (90%) cells: fat spaces.

Hematopoiesis :

Depressed with depressed granulopoieis in count and maturation, depressed erythropoiesis , depressed megakariopoiesis in count, maturation and topography.

Myeloid erythroid ratio :

.

Abnormal infiltration :

Diffuse infiltration with plasma cells.

Fibrosis :

Nill

Trabeculae :

Normal

Blood vessels :

Normal

Bone Marrow Aspiration
[[DOCTYPE:BONE_MARROW_ASPIRATION]]

Patient Name	**: فاتن على السيد**	**Patient Id**	**: 1905**
Patient Info.	**: Female 69Y**	**VisitId**	**: 359561**
Department	**: Out Patient**	**Collection Date**	**: 22/08/2026 09:45:12**
Profile NO.	**: 2327-2025 /25610100104609 / 11891**	**Reporting Date**	**: 03/09/2026 12:55:31**

Bone Marrow Biopsy

CD138: 80%

Diagnosis and conclusion :

Referred patient as MM post BMT , BMB is markedly hyper cellular for age , showing diffuse infiltration with plasma cells 80 % highlighted with CD138 .
Patient is in morphologic relapse .

Recommendation :

For Clinical Correlation

Dr. Hend Abd Elhady, M.D
Ass. Prof. of Hematopathology,
NCI
Cairo University

Bone Marrow Aspiration
[[DOCTYPE:BONE_MARROW_ASPIRATION]]

Name : Faten Ali El-Sayed	Date : 06-09-2026 09:32
Branch : Nasrcity	Medical No. : 308455
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : التامين الصحي

Follow up whole body (FDG) PET/CT Study:

Clinical history:

- 70-year-old female patient with history of managed MM, PET/CT dated 04/02/2026 is available for comparison and referred for follow up.

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROI.s and the normal threshold is **< 3.0**

PET/CT findings:

Compared to PET/CT dated 04/02/2026:

Head and neck:

- Bilateral symmetrical tracer distribution is seen within the brain parenchyma with no abnormal metabolic activity or definite SOLs could be detected.
- Normal appearance of soft tissues and glandular structures of head & neck.
- No pathologically enlarged or tracer-avid cervical or supraclavicular LN.s.

Chest:

- No current significant metabolically active lung nodules or parenchymal infiltrates.
- No enlarged or tracer-avid mediastinal, hilar or axillary LN.s.
- No pleural thickening, masses or effusion.

Abdomen and Pelvis:

- **Newly developed metabolically active epigastric/pancreatic tail region soft tissue lesion with SUV max 10.5.**
- Average-sized liver showing homogenous CT-texture and tracer uptake pattern with no definite focal lesions (on non-contrast CT basis.
- Unremarkable spleen, kidneys, adrenals and pelvic organs.

NILE DIAGNOSTIC CENTER

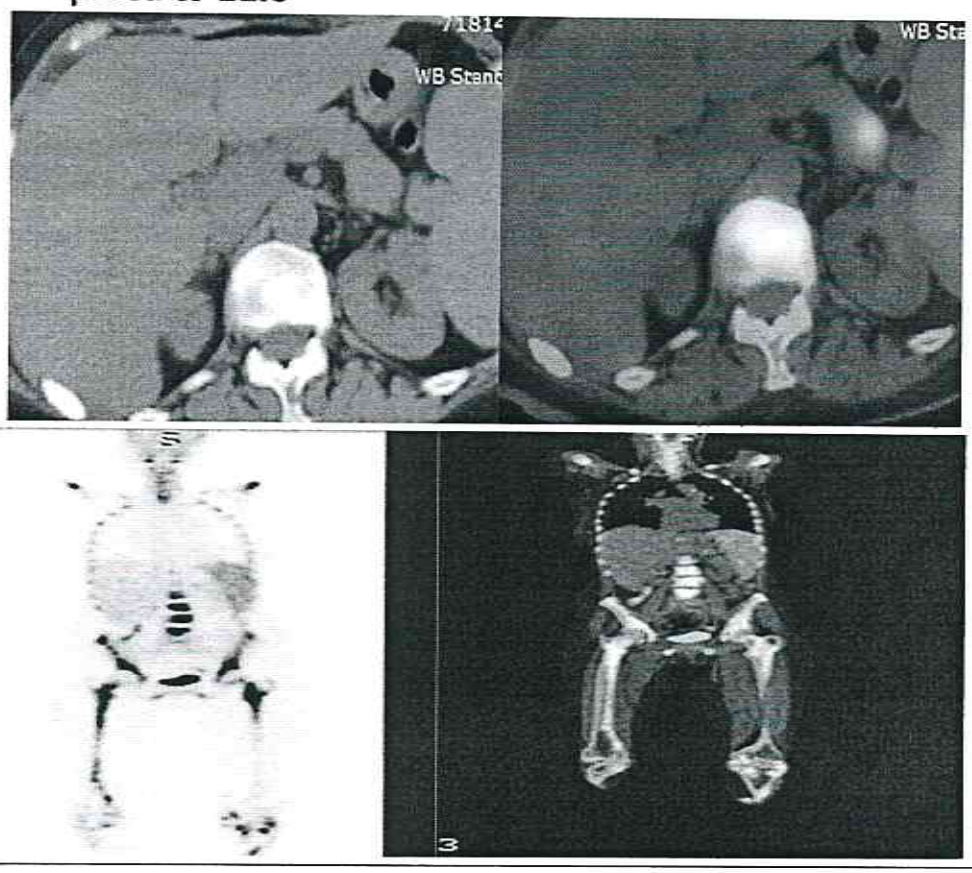
PET Scan
[[DOCTYPE:PET_SCAN]]

Name : Faten Ali El-Sayed	Date : 06-09-2026 09:32
Branch : Nasrcity	Medical No. : 308455
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : التامين الصحي

- No definite gastric or bowel thickening, mural masses (on non-oral contrast CT basis) or tracer-avid foci could be detected.
- No pathologically enlarged or tracer-avid abdomino-pelvic LN.s.
- No evidence of ascites, free or encysted abdomino-pelvic fluid collections.

Musculoskeletal & BM:

- **Progressive course of the previously noted wide spread osseous lesions at the most of axial and appendicular skeleton with SUV max 14.5 compared to 11.0**



NILE DIAGNOSTIC CENTER

Name : Faten Ali El-Sayed	Date : 06-09-2026 09:32
Branch : Nasrcity	Medical No. : 308455
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : التامين الصحي

Impression:

Follow up Whole-body (FDG) PET/CT study is showing:

- **Progressive course of the previously noted metastatic osseous lesions.**
- **Newly developed metabolically active epigastric/pancreatic tail region soft tissue lesion as described for MRI correlation.**
- **The rest of the surveyed body show no scintigraphic evidence of any other significant metabolically active lesions.**

Best regards,

Dr. Ahmed dowydar, MSc.

Dr. Alaa Nagi, MD.
Nuclear Medicine

PET Scan
[[DOCTYPE:PET_SCAN]]

Patient Name	**: فاتن على السيد**	**Patient Id**	**: 1905**
Patient Info.	**: Female 69Y**	**VisitId**	**: 359561**
Department	**: Out Patient**	**Collection Date**	**: 22/08/2026 09:45:12**
Profile NO.	**: 2327-2025 /25610100104609 / 11891**	**Reporting Date**	**: 23/08/2026 10:23:36**

Bone Marrow Aspiration

Bone Marrow Differential Count

Normoblasts :	65	Blasts :	0
Promyelocytes :	1	Myelocytes :	1
Juvenile :	0	Staff :	5
Segment :	7	Lymphocyte :	14
Monocyte :	2	Eosinophil :	0
Basophil :	0	Plasma cell :	5

Peripheral blood comment :

Pancytopenia

Bone Marrow Aspiration / Touch Print Comment

Cellularity :

Hypocellular marrow for age

Erythropoiesis :

Normoblastic reaction with dysplasia

Granulopoiesis :

Depressed granulopoiesis in count

Thrombopoiesis :

Depressed thrombopoiesis

Lymphopoiesis and plasma cells :

Normal lymphocytes.

Others :

-

Abnormal Infiltration :

5% plasma cells

Conclusion :

Referred patient as myeloma post BMT, bone marrow aspiration is hypocellular for age, with mild increase in plasma cells

Recommendations :

Correlation with BMB and IHC.

Dr. Hend Abd Elhady, M.D
Ass. Prof. of Hematopathology,

Bone Marrow Aspiration
[[DOCTYPE:BONE_MARROW_ASPIRATION]]

Patient Name	**: فاتن على السيد**	**Patient Id**	**: 1905**
Patient Info.	**: Female 69Y**	**VisitId**	**: 373525**
Department	**: Out Patient**	**Collection Date**	**: 09/09/2026 12:32:14**
Profile NO.	**: 2327-2025 /25610100104609 / 11891**	**Reporting Date**	**: 09/09/2026 13:05:20**

Clinical Chemistry Report

Test		Result	Unit	Reference Range
Kidney Function				
Blood Urea	: **H**	64.6	mg/dL	18.6 - 52.8
Serum Creatinine	: **H**	1.35	mg/dL	0.5 - 0.9
Calcium Total	:	9.25	mg/dL	8.8 - 10.6
Calcium Ionized (Ca++)	:	1.22	mmol/L	N. 1.1 - 1.4
Liver Function				
Total Bilirubin	: **H**	2.42	mg/dL	0.1 - 1.2
Direct Bilirubin	: **H**	0.66	mg/dL	0 - 0.3
SGOT (AST)	:	14	U/L	0 - 35
SGPT (ALT)	:	14	U/L	0 - 34

Signature

Dr. Reaam Abdelmonem

[[DOCTYPE:LAB_TEST]]

Patient Name	: فاتن على السيد	Patient Id	: 1905
Patient Info.	: Female 69Y	VisitId	: 359561
Department	: Out Patient	Collection Date	: 22/08/2026 09:45:12
Profile NO.	: 2327-2025 /25610100104609 / 11891	Reporting Date	: 03/09/2026 12:55:31

Bone Marrow Biopsy

Clinical Data :

Referred patient as MM .

Peripheral blood comment :

Pancytopenia

Bone marrow aspiration comment :

Hypocellular for age ,5% plasma cells

Nature of specimen :

Bone marrow biopsy

Gross Examination :

BMB core from R/PSIS, 1.5 cm, unilateral

Technique :

Sections were prepared from paraffin block, processed in Bench Mark ultra-machine (Ventara), and stained with rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain.
the sections treated against:

Microscopic Examination.

Adequacy :

Adequate more than 7 inter-trabecular spaces.

Cellularity :

Marked Hyper cellular for age (90%) cells: fat spaces.

Hematopoiesis :

Depressed with depressed granulopoieis in count and maturation, depressed erythropoiesis , depressed megakariopoiesis in count, maturation and topography.

Myeloid erythroid ratio :

Abnormal infiltration :

Diffuse infiltration with plasma cells.

Fibrosis :

Nill

Trabeculae :

Normal

Blood vessels :

Normal

Bone Marrow Aspiration
[[DOCTYPE:BONE_MARROW_ASPIRATION]]

Bone Marrow Aspiration
[[DOCTYPE:BONE_MARROW_ASPIRATION]]

Patient Name	**: فاتن على السيد**	**Patient Id**	**: 1905**
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Bone Marrow Biopsy

IHC :

CD138: 80%

Diagnosis and conclusion :

Referred patient as MM post BMT , BMB is markedly hyper cellular for age , showing diffuse infiltration with plasma cells 80 % highlighted with CD138 .
Patient is in morphologic relapse .

Recommendation :

For Clinical Correlation

Dr. Hend Abd Elhady, M.D
Ass. Prof. of Hematopatholo
NCI
Cairo University



Patient Name	**: فاتن على السيد**	**Patient Id**	**: 1905**
Patient Info.	**: Female 69Y**	**VisitId**	**: 359561**
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Myeloid erythroid ratio :

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Abnormal infiltration :

Diffuse infiltration with plasma cells.

Fibrosis :

Nill

Trabeculae :

Normal

Blood vessels :

Normal

Bone Marrow Aspiration
[[DOCTYPE:BONE_MARROW_ASPIRATION]]

Patient Name	**: فاتن على السيد**	**Patient Id**	**: 1905**
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Bone Marrow Biopsy

CD138: 80%

Diagnosis and conclusion :

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Patient is in morphologic relapse .

Recommendation :

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Cairo University

Bone Marrow Aspiration
[[DOCTYPE:BONE_MARROW_ASPIRATION]]

Name : Faten Ali El-Sayed	Date : 06-09-2026 09:32
Branch : Nasrcity	Medical No. : 308455
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : التامين الصحي

Follow up whole body (FDG) PET/CT Study:

Clinical history:

- 70-year-old female patient with history of managed MM, PET/CT dated 04/02/2026 is available for comparison and referred for follow up.

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROI.s and the normal threshold is **< 3.0**

PET/CT findings:

Compared to PET/CT dated 04/02/2026:

Head and neck:

- Bilateral symmetrical tracer distribution is seen within the brain parenchyma with no abnormal metabolic activity or definite SOLs could be detected.
- Normal appearance of soft tissues and glandular structures of head & neck.
- No pathologically enlarged or tracer-avid cervical or supraclavicular LN.s.

Chest:

- No current significant metabolically active lung nodules or parenchymal infiltrates.
- No enlarged or tracer-avid mediastinal, hilar or axillary LN.s.
- No pleural thickening, masses or effusion.

Abdomen and Pelvis:

- **Newly developed metabolically active epigastric/pancreatic tail region soft tissue lesion with SUV max 10.5.**
- Average-sized liver showing homogenous CT-texture and tracer uptake pattern with no definite focal lesions (on non-contrast CT basis.
- Unremarkable spleen, kidneys, adrenals and pelvic organs.

NILE DIAGNOSTIC CENTER

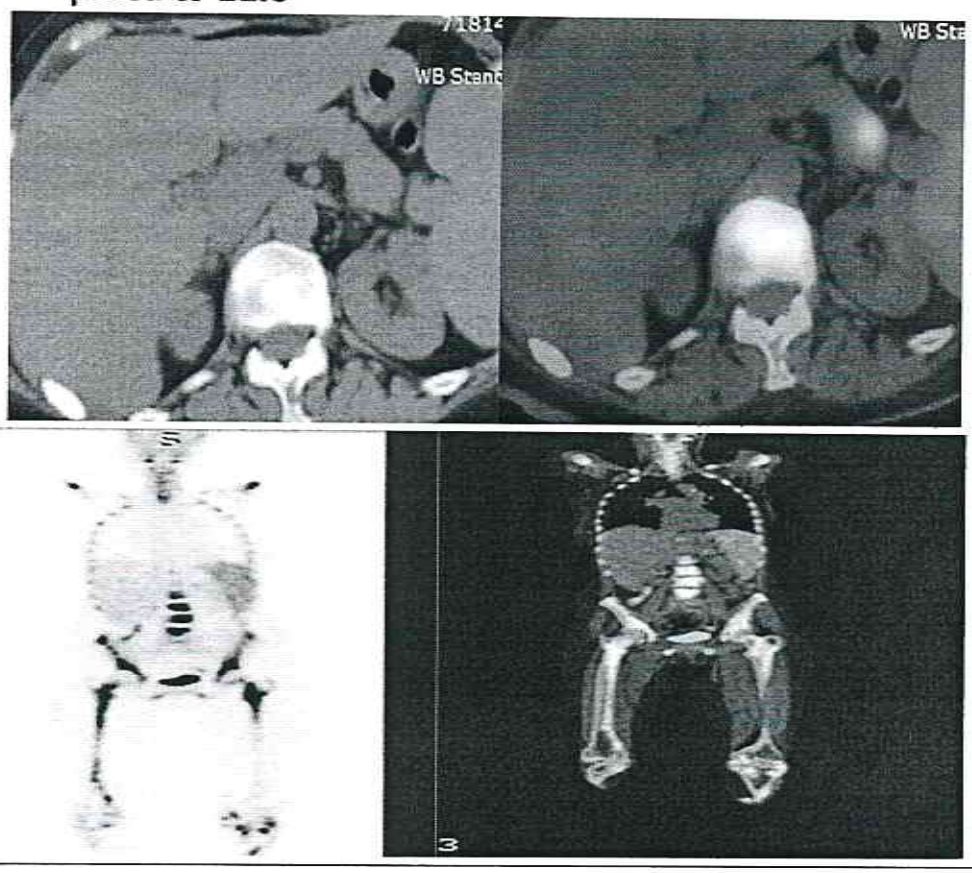
PET Scan
[[DOCTYPE:PET_SCAN]]

Name : Faten Ali El-Sayed	Date : 06-09-2026 09:32
Branch : Nasrcity	Medical No. : 308455
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : التامين الصحي

- No definite gastric or bowel thickening, mural masses (on non-oral contrast CT basis) or tracer-avid foci could be detected.
- No pathologically enlarged or tracer-avid abdomino-pelvic LN.s.
- No evidence of ascites, free or encysted abdomino-pelvic fluid collections.

Musculoskeletal & BM:

- **Progressive course of the previously noted wide spread osseous lesions at the most of axial and appendicular skeleton with SUV max 14.5 compared to 11.0**



NILE DIAGNOSTIC CENTER

Name : Faten Ali El-Sayed	Date : 06-09-2026 09:32
Branch : Nasrcity	Medical No. : 308455
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : التامين الصحي

Impression:

Follow up Whole-body (FDG) PET/CT study is showing:

- **Progressive course of the previously noted metastatic osseous lesions.**
- **Newly developed metabolically active epigastric/pancreatic tail region soft tissue lesion as described for MRI correlation.**
- **The rest of the surveyed body show no scintigraphic evidence of any other significant metabolically active lesions.**

Best regards,

Dr. Ahmed dowydar, MSc.

Dr. Alaa Nagi, MD.
Nuclear Medicine

PET Scan
[[DOCTYPE:PET_SCAN]]